



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : هبه محمد محمد نسيم

العنوان : ...

الحى / المدينة : حلوان

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 07-10-2026

رقم البطاقة : 27405200104384 رقم التليفون : 01116080169

العمر (تاريخ الميلاد) : 20-05-1974 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
التوقيع: (-----)

الإحصائي الاجتماعي
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني	الوظيفة: -----	التوقيع: -----
الاسم: د/ندى حسن	الوظيفة: -----	التوقيع: -----
الاسم: د/نرمين رمضان	الوظيفة: -----	التوقيع: -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: هبة محمد محمد نسيم National ID: 27405200104384	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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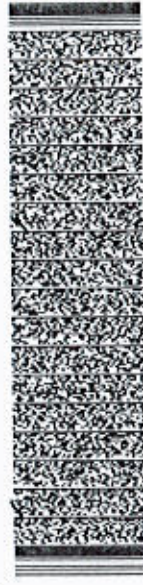
وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٢٠٢٣/١٠
٢٧٤.٥٢.٠١٠٤٣٨٤
حاصلة على بطوينة تسمى قس تجارى
انلى مسمية
عصام لطفى حسانين احمد
البطاقة سارية حتى ٢٠٢٣/١٠/١٧
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هبة
محمد محمد نسيم
٦ شى الخفاء الرشدين - الملاوة
حوران - القاهرة
٢٠٢٣.٠١.٢٠ ٢٠٢٤.٠٥.١٤
Kf7887274




Patient Name: Heba Mohamed Mohamed Nasim	Date : 01/11/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 51 Year(s)
Medical ID: 2094/2024	Pat. ID: 49255

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings: compared to the last study dated 30/7/20205

I Chest:

- Right mastectomy with clear operative bed , no masses or collection .
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUD seen in uterus.
- Left adnexal cyst no barely appreciated .
- Normal appearance of the other pelvic organs.
- Minimal pelvic ascites.
- Right iliac wing sclerotic and manubrium sterni lytic lesions the former is measuring 1 cm.
- D12 upper VEP subchondral sclerotic patch is seen mostly degenerative
- Newly noted sacral sclerotic lesion.

Opinion:

- Newly noted sacral sclerotic lesion for correlation with bone scan.
- Rather stationary rest of the study

DR.Reham.Khader

Patient Name: Heba Mohamed Mohamed Nasim	Date : 01/11/2025
Exam : ultrasound ABDOMEN& PELVIS	Age : 51 Year(s)
Medical ID: 2094/2024	Pat. ID: 49255

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Enlarged in size (17cm at Rt MCL) bright parenchymal echo pattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Not distended enough however no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected

Opinion:

Bright hepatomegaly likely fatty
Marked colonic gaseous distention

Signature Dr . Nermien Ramadan
PM 12:34 2025/11/01

Patient Name : الأستاذة / رانيا محمد فهد
Patient No. : 27405200104384
Path No. : 1902/24
Sex : Female
Age : 50 Y
Referring Doc :

Request No. : 13003896
Request Date : 05/06/2024
Verification Date : 12/06/2024
Financial Category : مبالغه - مبالغه
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast lesion.

Nature of Specimen: Tru-cut biopsy, collected at 4/06/2024.

Gross Examination:

- Multiple grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:

- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- Ductal carcinoma in situ: No.
- Perineural invasion: No.
- Angiolymphatic invasion: is inconspicuous.

Diagnosis:

Right breast lesion, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Rana Fathy.
Dr. Sagda Elkady.

Verified by :

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Room No. (4) - First Floor-Building No. 987. Nile Corniche - Miss El Qadima - Cairo, Egypt



Patient Name : الأمينة / دية محمد محمد سعيد
Patient No. : 27405200104384
Path No. : 1902/24
Sex : Female
Age : 50 Y
Referring Doc. :

Request No. : 13003896
Request Date : 05/06/2024
Verification Date : 01/07/2024
Financial Category : أورام دار السلام - مبانى
Department : Out Patient
IHC No. :
Registered by : Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 4/6/2024

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

IPT (Positive control showed appropriate reactivity):

ER: No nuclear reaction in tumor cells (score 0/8).

PR: Weak nuclear reaction in 3% tumor cells (score 3/8).

Her2: Moderate complete membrane reaction in >10 % tumor cells (score 2+) for SISH

Ki67: 30% nuclear reaction of tumor cells.

Conclusion:

Right breast cancer, Tru-cut biopsy:

Invasive duct carcinoma, Grade II

PT:

ER: Negative (score 0/8).

PR: Positive(score 3/8).

Her2: Equivocal (score 2+) for SISH

Ki67: Positive in 30% of tumor cells.

Scoring system: "ER, PR"

SCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Miskr El Qadima - Cairo, Egypt

Patient Name الأستاذة / هبة محمد محمد نسيم
Patient No. 27405200104384
Path No. 1902/24
Sex Female
Age 50 Y
Referring Doc.

Request No. 13003896
Request Date 05/06/2024
Verification Date 01/07/2024
Financial Category برنامج دار السلام - مياثره
Department Out Patient

Registered by Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Rana Fathy
Dr. Heba Hassan

Verified by :

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mitr El Qadema - Cairo, Egypt



Patient Name: Heba Mohamed Mohamed Nasim	Date : 30/07/2024
Exam : C.T CHEST WITH CONTRAST	Age : 50 Year(s)
Medical ID: 2094/2024	49255

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:

- Right breast edema , skin thickening (1.4 cm) with multiple sub cm ipsilateral axillary lymph nodes.
- Few bilateral tiny pulmonary nodules reaching 4 mm, likely non-specific.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or



Patient Name: Heba Mohamed Mohamed Nasim	Date : 30/07/2024
Exam : C.T CHEST WITH CONTRAST	Age : 50 Year(s)
Medical ID: 2094/2024	49255

- IUD seen in uterus.
- Left adnexal cyst measures about 3.2x2 cm for sonographic assessment.
- Normal appearance of the other pelvic organs.
- Minimal pelvic ascites.
- Right iliac wing sclerotic and manubrium sterni lytic lesions the former is measuring 1 cm.
- D12 upper VEP subchondral sclerotic patch is seen mostly degenerative

Opinion:

- Few bilateral pulmonary nodules for follow up.
- No hepatic deposits.
- Few osseous lesions for bone scan.

Dr. Shaimaa Megahed, MSc

Signature Dr. Mohamed Mohsen

04/08/2024-06:29

PATIENT NAME: HEBA MOHAMMED MOHAMMED NASEEM

GENDER: Female

DATE : 16-Jul-2024 02:45 PM

PATIENT ID: 10124245401

AGE: 50 y

ORDER: 2941737

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected all over the surveyed skeleton.

Degenerative changes are seen over dorso-lumbar vertebrae.

Arthritic changes are seen over both knees.

Traumatic rib lesion is suggested at the right tibial bone.

Diffuse right breast uptake.

Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

18-Jul-2024 10:04 AM

PATIENT: HEBA MOHAMMED MOHAMMED NASEEM, 10124245401, Female, 50 y

Report version 1, 18-Jul-2024 10:04 AM

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Patient Name: Heba Mohamed Nasim	Mohamed	Date : 21/04/2024
Exam : MAMMOGRAPHY BOTH		Age : 50 Year(s)
Medical ID: 27405200104384		

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C).
- Right breast LIQ distortion by US 3.00 hypoechoic avascular distortion measures 11 x 19 mm with focal skin thickening.
- No left suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed no left solid or cystic mass lesions.
- Normal left skin thickness and nipple-areola complex.
- Nonspecific left axillary LNs.
- Right axillary pathological subcentimetric lymph nodes.

Opinion:

- Right breast LIQ distortion and pathological axillary lymph nodes (BIRADS 4c) for biopsy.

Yours truly

Dr.Wafaa Ali